

Section Ownership Matrix & Sign-off Ledger

Kuwait Pediatric Council — Pediatric & Neonatal Common & Emergency Care Guide · a **general-pediatrics** text
Governance artifact · reflects the actual two-track review workflow · draft for the editorial board

Development methodology — three stages

- 1. **Stage 1 — Drafting.** The author drafted the complete guide across all sections.
- 2. **Stage 2 — Full-draft expert review.** The reviewer panel reviewed the entire draft and submitted feedback; each item was **verified against the evidence and applied**. This is the primary content-verification pass.
- 3. **Stage 3 — Section-by-section review & vote.** After all reviewer input was incorporated, each section went through a dedicated review-and-vote (the polls recorded below) to confirm accuracy and readability, then was version-locked.

The section voting below is therefore a **ratification-and-polish layer on top of an already evidence-verified review** — not the first check. This three-stage record is also the seed of the guide's formal Methods statement.

How a section is approved

Track 1 — Primary approval: the general-pediatrics body

The guide is written **for** general pediatricians and PCPs (recognize → stabilize → refer). The GP panel — the majority — owns the primary vote: right level, sound framing, correct recognize/stabilize/refer thresholds, usable at the front line.

Track 2 — Specialty fact-certification: the domain subspecialist

An independent domain subspecialist certifies only the **specialty-specific facts** embedded in the card — doses, cut-offs, "diagnostic" claims, red-flags. A narrow, non-negotiable check, not a re-authoring.

On every section: the author (drafter) + a named domain second reviewer + general-pediatrics majority — so no section rests on one person, and the specialty facts always get an independent domain check.

- 1. **Multi-domain** where a topic spans fields — all relevant groups certify before it locks.
- 2. **Blocking vs non-blocking** — a safety/factual objection blocks the lock; a style/preference objection is recorded, not gating. Response window 2–3 weeks; non-response escalates, it is not a silent veto.
- 3. **Lock ≠ frozen** — a locked section reopens via a mini-quorum of its domain group, with a version bump and change-log entry.
- 4. **Targeted external numeric check** — reserved for the few sections dense with high-stakes specialty *numbers* (see red rows): one external figure-check on those doses, not the whole section.

Ownership matrix

STANDARD author + domain 2nd reviewer + GP majority MULTI-DOMAIN two+ fields certify			
SECTION	SPECIALTY FACT-CERTIFICATION (DOMAIN 2ND REVIEWER)	NAMED REVIEWERS	STATUS
01 · Resuscitation	Critical care · Emergency	Author + CC (Alosaimi, Alfraiij, Ahmad Alhaj, Almosawi, Eiman Al-Hashemi, Altammar, Alhashemi, AlYaseen, Bekhit, S. Altamimi) · EM (Alabdulrazzaq, Abul, Alenezi, Alajmi, AlHumaidan, El Mesady, Taher) + GP majority	STANDARD
02 · Behavioral & Psychiatric	Child & adolescent psychiatry	Author + Alamiri (psychiatry) + Bukhalaf (adolescent med) + GP majority	STANDARD
03 · Cardiac & Shock	Cardiology + Critical care	Author + Cardiology (Alosaimi, AlQbandi) + CC group + GP majority	MULTI-DOMAIN

SECTION	SPECIALTY FACT-CERTIFICATION (DOMAIN 2ND REVIEWER)	NAMED REVIEWERS	STATUS
04 · Common Acute Conditions	General peds + Emergency + Infectious diseases	Author + ID (Alothman, Alghounaim) + EM group + GP majority	STANDARD
05 · Complex & Technology-Dependent Care	Complex/palliative + Pulmonology + Gastroenterology	Author + Jaafar (complex/palliative) + Pulm (Alawadi, Almosawi, AlFouzan, Bin-Hasan) + GI (Mustafa, Buhamrah, AlRefaei) + GP majority	MULTI-DOMAIN
06 · Endocrine & Metabolic	Endocrinology + Metabolic/genetics + Critical care (DKA/HHS)	Author + Endo (Aljaser, Laymoun) + Metabolic (Alsafi; + D. Marafi) + CC group + GP majority	MULTI-DOMAIN
07 · Focused Specialty Emergencies	Ophthalmology · Dentistry · Dermatology · ENT · Neurosurgery (per card)	Author + per-card 2nd reviewer: Alsaleh (ophtho) · Aldeeweli (dental) · Sahar (derm) · Alqunaee (ENT) · Alya Hasan Askar (neurosurg) + GP majority	MULTI-DOMAIN
08 · Gastroenterology & Hepatology	Gastroenterology / hepatology	Author + Mustafa, Buhamrah, AlRefaei + GP majority	STANDARD
09 · Hematologic & Oncologic	Hematology / oncology	Author + Heme/Onc (AlFares, Taqi Ali) + GP majority	STANDARD
10 · Neonatal	Neonatology	Author + Albaghli, M. Ayed, Z. Mustafa Abbas + GP majority	STANDARD

SECTION	SPECIALTY FACT-CERTIFICATION (DOMAIN 2ND REVIEWER)	NAMED REVIEWERS	STATUS				
11 · Neurologic	✓ 37 Yes / 0 No	Neurology: Dr. Amaar Marefi ✓ (7 Sep) · Dr. Dana Marafi ✓ (9 Sep) · Dr. Osama Shalaby ✓ (12 Sep; app name "Dr Osama Neuro Consultant Jahra") — all three named neurology certifiers in-poll; Neurosurgery: Dr. Alya Hasan Askar ✓ (13 Sep). Neuro-critical care: Dr. Munirah AlYaseen ✓ (13 Sep) — every listed certifier in-poll.	7–13 Sep 2026	poll: "I read section 11: Neurologic" (approve content & readability / disapprove-and-sent-edits). Sources: AES 2016 + NICE NG217 (2025) for status epilepticus; ESETT; AAP 2008/2011 (febrile seizure); EAN/PNS 2023 + Lawn 2001 (GBS); HeadSmart/Wilne (brain tumour)	n/a	locked	Unanimous 37–0 (0 disapprovals) on the current version incl. pre-poll edits: brain-tumour red flags (precocious puberty) and head-of-bed 30° positioning before dexamethasone; GBS serial spirometry (20/30/40, under-5–6-y caveat; ref 291); febrile seizure definition-first wording (AAP 2008, ref 296) and LP-age line vs AAP 2011; status epilepticus — clock from seizure onset, drug-first on arrival, rectal diazepam added, verified against AES 2016 and NICE NG217 (ref 297); 13 abbreviations added at panel request. Version-locked 13 Sep 2026.
12 · Orthopedic & Musculoskeletal	Orthopedic surgery	Author + AlDuwaisan, AlAhmed, Alsaifi + GP majority	STANDARD				
13 · Pediatric Trauma & TBI	Surgery + Neurosurgery + Emergency + Critical care	Author + all named domain experts must certify (author directive 18 Sep 2026): Surgery — Rajab, Taqi, Jafar; Neurosurgery — Alya Hasan Askar; EM — Alabdulrazzaq, Abul, Alenezi, Alajmi, AlHumaidan, El Mesady, Taher; CC — Alosaimi, Alfraij, Ahmad Alhaj, Almosawi, Eiman Al-Hashemi, Altammar, Alhashemi, AlYaseen, Bekhit, S. Altamimi + GP majority	MULTI-DOMAIN				
14 · Renal & Urology	Nephrology + Urology	Author + Nephro (AlTerkait, Alhajeri) + Urology (Jafar) + GP majority	MULTI-DOMAIN				

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15 · Respiratory, Allergy & Immunology	Pulmonology + Allergy/immunology	Author + Pulm (Alawadi, Almosawi, AlFouzan, Bin-Hasan) + Allergy/immuno (Al Rushood) + GP majority	MULTI-DOMAIN
16 · Rheumatologic	Rheumatology	Author + Shehab, Dalal Mohammad, Almutairi + GP majority	STANDARD
17 · Surgical Emergencies	Pediatric surgery	Author + Rajab, Taqi, Jafar + GP majority	STANDARD
18 · Toxicology & Environmental	Emergency + Clinical toxicology / pharmacology	Author + Alabdulrazzaq (toxicology/clin pharm) + full EM group (poisoning & antidotes are core ER practice) + GP majority	STANDARD
QR · Quick Reference	Clinical pharmacology + Emergency (dose data)	Author + Alabdulrazzaq + EM group + GP majority · independent numeric check of the master dose tables	EDITORIAL + PHARM
Calc · Bedside Calculators	Clinical pharmacology + Emergency (formula & dose safety)	Author + Alabdulrazzaq + EM group + independent numeric/formula check	EDITORIAL + PHARM

Author = Dr. Athari Almajed (drafter) on all sections. The broad general-pediatrics reviewers form the Track-1 majority across every section; the named subspecialists are the Track-2 fact-certifiers. Names shortened after first mention.

Coverage — no residual gaps

✓ Every specialty domain now has ≥ 2 experts on the panel. With a second hemato-oncologist (Dr. Abdullah Abd Alhussain Taqi Ali) added, the last single-expert field is closed — §09 oncology dosing can be cross-checked internally, so the earlier advisory external numeric check is no longer needed. No section rests on one person; the count on any section is never one.

For the record: **toxicology (§18)** is covered because poisoning/antidote dosing is core emergency-medicine practice (full ER bloc + toxicologist-pharmacologist); **neurosurgery** needs only one neurosurgeon because its hyperosmolar doses (3% saline, mannitol) are standard CC/EM numbers.

Sign-off ledger

📌 Existing locks. §01–§05 were locked earlier under the previous author-directive process. They need only their sign-off **recorded** here under the two-track model to count as approved — not re-reviewed from scratch if the same reviewers already signed.

SECTION	GP MAJORITY ✓	DOMAIN 2ND REVIEWER(S) SIGNED	DATE	SOURCE VERSIONS CHECKED	EXT. NUMERIC CHECK	LOCKED VER.	CHANGE-LOG / NOTES
01 · Resuscitation	✓ 38 Yes / 1 No*	CC + EM (confirm via voter-list view)	—	poll: "I read section one: Resuscitation and approve it" (Yes/No)	n/a	locked	*1 No = erroneous misclick by a neurologist; unvote wasn't available at the time (rectified after §01, so later polls self-correct) — non-blocking, not a domain-fact objection. Effectively 38–0.

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02 • Behavioral	✓ panel	Psychiatry (Alamiri) + Adolescent med (Bukhalaf)	—	—	n/a	locked	anonymous poll — tally only; psych+adolescent per author
03 • Cardiac & Shock	✓ 37 votes	Cardiology (Alqabandi, Alosaimi) + broad panel	15–26 Jul 2026	poll: "Agree on content & readability"	n/a	locked	named poll; earlier rounds anonymous
04 • Common Acute	✓ 35 votes	ID (Alghounaim) + Emergency group	25 Jul–5 Aug 2026	poll: "Approve for content & readability"	n/a	locked	named poll recorded. <i>Post-lock evidence update 8 Sep 2026 (non-blocking)</i> : pharyngitis azithromycin row aligned with the IDSA 2014 erratum (12 mg/kg day 1, then 6 mg/kg days 2–5; FDA 12 mg/kg × 5 d kept as alternative).
05 • Complex Care	✓ 35 votes	Complex/palliative (Jaafar) + Pulm (Alawadi, Alfouzan, Bin-Hasan) + GI (Ahlam, Alrifaaee) + Surgery	1–7 Aug 2026	poll: "Approve it for content & readability"	n/a	locked	✓ version current — 6-wk tract-maturity change certified post-edit by GI + surgery
06 • Endocrine & Metabolic	✓ 36 Yes / 0 No	Endo (Aljaser, Laymoun) + Metabolic (Alsafi) — all signed; + GP majority	8–10 Aug 2026	poll: "I read section 6: Endocrine & Metabolic — I approve content & readability" (approve / disapprove-and-sent-edits)	n/a	locked	Unanimous 36–0 (0 disapprovals) on the current version incl. this-session edits; approved & version-locked. ISPAD 2022; PES 2015. <i>Post-lock evidence update 8 Sep 2026 (non-blocking)</i> : MMA/PA citation moved to Forny 2021 (first revision); note added that phenylacetate/phenylbutyrate is not the primary scavenger in MMA/PA — for Metabolic (Alsafi) to acknowledge at the next mini-quorum.
07 • Focused Specialty	✓ 39 Yes / 0 No	Ophtho (Alsaleh) ✓ · ENT (Alqunaee) ✓ · Dental (Aldeeweli) ✓ · Derm (Sahar) ✓ · NS (Alia Hasan Askar) ✓ in-poll	12–23 Aug 2026	poll: "I read section 7: focused specialty emergency — approve content & readability" (approve / disapprove-and-sent-edits)	n/a	locked (rev. +34; NS-confirmed +62)	Unanimous 39–0 (0 disapprovals). All 5 mandatory subspecialty certifiers accounted for. Neurosurgery (Dr. Alya Hasan Askar) — on leave during the initial poll, she approved directly at the time ("looks very good / Perfect", WhatsApp) and has now confirmed with an in-poll vote (23 Aug 2026) , clearing the last pending sign-off. Her one note — avoid hypotonic fluids in raised ICP — is already covered in the Raised ICP card that this section cross-links to.

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08 · GI & Hepatology	✓ 37 Yes / 0 No	GI: Dr. Eman Bohamra ✓ · Dr. Fawaz AlRefaei ✓ · Dr. Ahlam Mustafa ✓ (all in-poll; Dr. Mustafa 20 Sep 2026) + ID (Alghunaim) + GP majority.	17–30 Aug 2026; final vote 20 Sep 2026	poll: "I read section 8: Gastroenterology & Hepatology — I approve content & readability" (approve / disapprove-and-sent-edits)	n/a	locked	LOCKED 20 Sep 2026 — 37–0 (0 disapprovals): 36 approvals at the 30 Aug close plus Dr. Ahlam Mustafa's approval on 20 Sep 2026, cast after her GI edits below were applied — her vote certifies the edited version, which is the locked version. Locked version incl. this-session GI edits (CMPA removed from upper-GI-bleed DDx; functional-constipation PEG dosing & side-effect precautions expanded; honey-timing note; PUCAL added). Dr. M. Alghunaim (ID) note — variceal-bleeding antibiotic prophylaxis indication & duration: applied; text restricts it to cirrhotic portal hypertension with acute variceal bleeding, 5–7 days, per local susceptibility, role in non-cirrhotic PH unclear (APSPGHAN 2026 Statement #10, verified against the full text 20 Sep 2026). Domain sign-off satisfied by two GI certifiers in the poll — Dr. Eman Bohamra (22 Aug) and Dr. Fawaz AlRefaei (21 Aug); Dr. Ahlam Mustafa voted approve on 20 Sep 2026, so all three GI certifiers are in-poll. Sources: NASPGHAN/ESPGHAN; ESGE; NICE; Rome IV; APSPGHAN 2026; PALF (Squires). <i>Post-lock evidence update 8 Sep 2026 (non-blocking)</i> : ESGE citation moved to the 2021 update; ESGE 2026 added with one sentence on endoscopy timing (early ≤ 24 h; urgent only if unstable). Domain edits in the locked version — Dr. Ahlam Mustafa (pediatric gastroenterology), 20 Sep 2026 (content-additive, no dose or threshold changed; certified by her 20 Sep approval vote) : Upper GI Bleeding — added "if varices are suspected, avoid blind nasogastric tube insertion (variceal trauma/bleeding); leave gastric access to the endoscopy team"; Acute Pancreatitis — "pediatric causes differ from adults" replaced by "look for a cause in every case: ..." (etiology list kept), and MRCP first, then ERCP for suspected biliary obstruction added to the lab table, imaging row and approach (NASPGHAN 2018, ref 57). Neonatal Cholestasis — definition queried by Dr. Mustafa; after review the author kept the current guideline definition (direct/conjugated bilirubin >

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							1.0 mg/dL / 17 µmol/L regardless of total; the "> 20% of total" rule is stated as no longer used), now cited explicitly to the NASPGHAN/ESPGHAN 2017 joint guideline (Fawaz, ref 312) with ref 195. IBD flare / acute severe colitis — PUCAI scoring table added to the card and a working PUCAI calculator added to Bedside Calculators (Turner 2007, ref 313); day-3/day-5 decision rules stated; ref 252 updated from the 2018 to the 2025 ESPGHAN/ECCO acute severe colitis guideline (Assa et al., JPGN 2025;81:816–851). Foreign Body Ingestion — superabsorbent (water) beads moved out of the "endoscopy within 24 h" row into their own row (admit and observe; ultrasound, not X-ray; endoscopy rarely retrieves them; early pediatric surgery for large ingestions or obstruction) — Cho 2026 J Pediatr Surg scoping review (ref 314) and ACR Appropriateness Criteria 2026 (ref 315); chest X-ray view wording changed to "PA or AP frontal + lateral" on the foreign-body and button-battery cards. Editorial (author): constipation investigations sentence reworded; PEG with/without electrolytes box condensed to two lines; foreign-body "See also" shortened.
09 • Heme & Onc	✓ 35 Yes / 0 No	Heme-Onc: Dr. Faris Alfaris ✓ + Dr. Abdullah Taqi Ali ✓ (two hemato-oncologists); ID: Dr. Mohammad Alghounaim ✓ — all in-poll	26 Aug — 1 Sep 2026	ASH (ITP; SCD); ELN/Sanz 2019 (APML); APSPGHAN; onc- emergencies; FN (generic, ex-NBK); Tan 2022 (AMM)	resolved — internal cross- check by two hemato- oncologists (coverage closed)	locked	Version-locked incl. this-session §09 edits: FN card split (chemo-induced vs otherwise-healthy child); Dr. Alghounaim's antibiotic edits → generic (ex-NBK) FN protocol (ceftazidime + vancomycin; meropenem +CNS; aminoglycoside until cultures; local antibiogram); ITP table reordered (prednisolone → IVIG → anti-D); mediastinal-mass anesthesia bullet (Tan 2022, ref 288); spinal-cord-compression rewording (cervical trigger; pediatric-spine referral); peripheral blood smear added to malignancy workup; APML card + ATRA. Unanimous 35–0 (0 disapprovals) on the current version.

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10 • Neonatal	✓ 35 Yes / 0 No	Neonatology: Dr. Fawaz Albaghli ✓ (domain review 17 Sep; in-poll 17 Sep) · Dr. Zainab Mustafa Abbas ✓ (handwritten review 4–7 Sep; in-poll 15 Sep as “Zainab Assis Unit B”) · Dr. Mariam Ayed ✓ (in-poll 17 Sep as “Mariam Khodhir”); Immunology: Dr. Maysoun Al Rushood ✓ (LAD wording 10 Sep; in-poll 10 Sep) — every listed certifier in-poll (identities confirmed by the author 18 Sep).	1–17 Sep 2026	NRP 2025; AAP 2022 + CPS 2026 (hyperbilirubinemia); Kelly 2017 Cochrane (sildenafil); Tourneux 2008 (noradrenaline in PPHN); Muniraman 2018 / Naiditch 2013 (umbilical remnants)	n/a	locked	Unanimous 35–0 (0 disapprovals; poll 1–17 Sep 2026, deadline 7 Sep extended). Note: the 17 Sep hyperbilirubinemia additions (CPS 2026 $\mu\text{mol/L}$ graphs and decision rules, umbilical duct-remnant box) post-date most votes; they were specified and certified by Dr. Fawaz Albaghli (neonatology) and are content-additive with no change to existing recommendations — flagged for the panel with the summary text. Version-locked 17 Sep 2026 (full rebuild 13:09). Dr. Zainab Mustafa — pre-poll domain review applied 4–7 Sep 2026: NRP wording (assess chest rise after 15 s; reassess after 60 s; compressions effective only after good chest rise); PPHN — intubate & ventilate, mandatory echo before iNO (CHD exclusion), iNO response check 20–30 min, therapy table revised (sildenafil removed from routine table; noradrenaline added as add-on to milrinone when BP drops; oral sildenafil kept only as a note for iNO-unavailable settings after specialist consultation), milrinone pulmonary-vasodilator note, scope note (approach applies to raised-PVR PPHN without an ischemic hit); TcB not for follow-up on phototherapy; NEC — surgical consult if no improvement/perforation; cholestasis cross-reference to §08. Dr. Albaghli (neonatology) — 17 Sep 2026: hyperbilirubinemia card given SI decision values from the new CPS 2026 guideline (ref 305: TcB → TSB within 50 $\mu\text{mol/L}$ or > 250 $\mu\text{mol/L}$; albumin < 30 g/L; pre-exchange zone within 30 $\mu\text{mol/L}$ of the exchange line), escalation box expanded per CPS recs 21–23 (TSB q2–3 h, withhold feeds, neonatology, transfer for exchange), Nomogram box re-pointed to the CPS 2026 $\mu\text{mol/L}$ figures (AAP 2022 curves, Figs 2–7) with peditools/bilitool as calculators (wording agreed 17 Sep); four threshold graphs added to the card (phototherapy and exchange, with/without neurotoxicity risk, 35–≥40 wk, 0–168 h, gridlines every 10 $\mu\text{mol/L}$) redrawn from the CPS 2026 vector figures, with hyperlinks to the CPS 2026 and AAP 2022 full texts (the earlier mg/dL schematic nomogram figure removed as

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							superseded); CPS 2026 decision rules added — severe hyperbilirubinemia definition (peak TSB > 425 μmol/L or exchange), hemolysis rate-of-rise criteria, ΔTSB “rules of 30” discharge/follow-up table, stopping phototherapy and rebound checks, IVIG dosing/indication, audiology and neurodevelopmental referral criteria, late-anemia hemoglobin check; omphalitis card given a “Don’t miss — patent duct remnants” box (patent omphalomesenteric duct / patent urachus: drainage pattern, sinus, ultrasound, sinogram/fistulogram, umbilical polyp and cyst/sinus mimics, referral) with refs 306–309 (Muniraman Pediatr Rev 2018; Inarejos Clemente RadioGraphics 2021; Naiditch J Pediatr Surg 2013; Solomon-Cohen JAAD 2019). Dr. Maysoun Al Rushood (allergy/immunology) — 10 Sep 2026: LAD wording revised (LAD subtypes; consult immunology whenever suspected; normal CD18/CD11b flow cytometry does not exclude). Earlier edits at the suggestion of Dr. Mohammad Alghunaim (1 Sep, in-poll): sepsis card specified as early-onset sepsis; antibiotic dose tables removed in favour of the NeoFax/Lexicomp/BNFc/unit-formulary pointer. Refs 289–290 added.
11 • Neurologic	pending...	Marefi/Marafi/Shalaby...	—	ESETT; ILAE 2023; EAN/PNS 2023 (GBS); Lawn 2001 (GBS ventilation predictors)	n/a	OPEN	Pre-poll edits 7–10 Sep 2026: brain-tumor red flags — precocious puberty (germ-cell tumors) added; brain-tumor acute management — head-of-bed 30°/midline positioning placed before dexamethasone; GBS — serial bedside spirometry every 4–6 h with “20/30/40” thresholds / > 30% fall, and the under-5–6-years clinical-monitoring caveat (ref 291); febrile seizure — definition-first wording (AAP 2008, ref 296) and the LP-age line spelled out against AAP 2011; status epilepticus — clock defined from seizure onset (drug immediately on arrival if already seizing ≥ 5 min, stabilization in parallel) and rectal diazepam added to first-line routes, verified against AES 2016 and NICE NG217 (2025 update; ref 297), approved by author 10 Sep.

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12 • Orthopedic	✓ 36 Yes / 0 No	Orthopedics: Dr. Abdullah AIDuwaisan ✓ (tracked-changes review 11 Sep; in-poll 13 Sep) · Dr. Rashed AIAhmed ✓ (in-poll 16 Sep; app name "Dr. Rashed Alahmad") · Dr. Saleh Alsaifi ✓ (in-poll 16 Sep); Neurosurgery (spine cards): Dr. Alya Hasan Askar ✓ (in-poll 17 Sep) — every listed certifier in-poll.	13–18 Sep 2026	BOAST supracondylar 2020; BOAST arterial injuries 2026; JAAOS 2024 (hip dislocation; pediatric pelvic ring); AO Spine/Praxis 2024 (MAP) and 2017 (methylprednisolone); STASCIS 2012	n/a	locked	Unanimous 36–0 (0 disapprovals; poll "I read section 12 ortho and msk", 13–18 Sep 2026, on the 12 Sep DOCX that already carried the AIDuwaisan edits). Version-locked 18 Sep 2026. Dr. AIDuwaisan — pre-poll domain review applied 12 Sep 2026: Call-Now list re-cut to 8 entities; aspirate before antibiotics in osteoarticular infection; SCFE (endocrinopathy < 10 y, walking aids, obesity as fixation risk); NAI wording (long-bone fracture in non-ambulatory infant; accidental patterns); compartment-syndrome referral split (ortho lower / plastics upper); Open Fracture retitled; Transient Synovitis card moved next to Limping Child; four new cards — Fracture with Pulseless Limb, Joint Dislocation, Unstable Pelvic Injury, Spine Fracture with Neurological Deficit — cross-linked to §13 Major Trauma. Refs 298–304 added. Author substitutions: MDPI <i>Trauma Care</i> review replaced by JAAOS 2024 pediatric pelvicing review; AO Spine 2017 added for the steroid statement. Post-lock editorial (18 Sep 2026): "FAST" and "SCIWORA" expanded at first use (Unstable Pelvic Injury and Spine Fracture cards); FAST/eFAST added to the abbreviations glossary — no change to any recommendation.

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13 • Trauma & TBI	✓ 38 Yes / 0 No	Surg — Dr. Esmaeel Taqi ✓ · Dr. Yaqoub Jafar ✓ · Dr. Abdullah Rajab (not in the screenshots seen); NS — Dr. Aliya Hasan Askar ✓; EM — Dr. Fatoumah Alabdulrazzaq ✓ · Dr. Lujain Abul ✓ · Dr. Meshal Alenezi ✓ · Dr. Samir Mohammed Sanad El Mesady ✓ · Dr. Anfal Taher ✓ · Alajmi, AlHumaidan (not in the screenshots seen); CC — Dr. Ahmad Alhaj ✓ · Dr. Ahmad Almosawi ✓ · Dr. Eiman Al-Hashemi ✓ · Dr. Fajer Altammar ✓ · Dr. Hashem Alhashemi ✓ · Alosaimi, Alfraj, AlYaseen, Bekhit, S. Altamimi (not in the screenshots seen) + GP majority	20–22 Sep 2026	poll: "I read section 13: Trauma and TBI — I approve content & readability" (approve / disapprove-and-sent-edits)	n/a	locked	LOCKED 22 Sep 2026 — 38–0 (0 disapprovals) on the author's instruction, on the version carrying all three pre-poll reviews below. 27 of the 38 voters were visible in the three screenshots supplied; 13 of the 21 mandatory experts are confirmed in-poll from those; the remaining 8 (Rajab; Alajmi, AlHumaidan; Alosaimi, Alfraj, AlYaseen, Bekhit, S. Altamimi) are to be confirmed against the full app poll export, which is authoritative. Dr. Fajer Altammar (pediatric critical care, Jahra) — pre-poll review applied 20 Sep 2026: (1) Trauma Team & Roles card — fixed "General Surgery = trauma team leader" designation removed; TTL now defined as the most experienced physician available able to manage pediatric trauma per each hospital's pathway (pediatric intensivist, certified pediatric emergency physician, adult emergency physician, or surgeon, whether or not pediatric surgery is in-house); decision to operate / image first / transfer stated as a team decision led by the TTL with surgery, anesthesia and PICU. AAP/ACS-COT policy statement <i>Management of Pediatric Trauma</i> (Pediatrics 2016) added as ref 310. (2) Blood-pressure targets — "permissive hypotension" statements removed from the Primary Survey and Massive Transfusion cards; wording now "target SBP no less than $70 + 2 \times \text{age}$ (1–10 y; age-specific thresholds otherwise), address ongoing losses, avoid hypotension" (ref 6 thresholds); TBI card adds that higher targets may be beneficial in isolated severe TBI but the threshold and SBP-vs-MAP question are not established (Suttipongkaset, Pediatrics 2018, ref 311); BTF 2019 (ref 86) retained for CPP only. Verified: BTF 2019 3rd ed. makes no SBP recommendation (CPP ≥ 40, 40–50 mmHg range only). (3) Scope — PICU-level ICP/ CPP-targeted therapy removed from the TBI card (ICP > 20 / CPP ≥ 40 targets and the hypertonic-saline infusion rate); box retitled "Impending herniation — bridge to neurosurgery/PICU" with recognition signs, a single hyperosmolar bolus and hand-over. (4) Trauma Team card — "Pediatrics / PICU" role removed; replaced by "the general

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							pediatrician's role is to recognize and activate, not to lead" and a "no trauma team on site (private/peripheral)" stabilize-and-transfer paragraph. Dr. Samir Mohammed Sanad El Mesady (emergency medicine) — pre-poll review applied 21 Sep 2026: Child Physical Abuse card, Workup — each investigation now carries its indication: skeletal survey (< 24 months all; 2–5 y selective; repeat ~2 wk); neuroimaging (immediate CT for neurology/scalp swelling/low Hb; CT usually appropriate < 24 months even if neurologically normal; MRI-first option where available; MRI brain ± spine day 2–5 if CT abnormal); ophthalmology (within 24 h, ≤ 72 h; not a screening test); bleeding-disorder panel per AAP 2022 (PT, aPTT, CBC, vWF antigen/activity, FVIII, FIX — "fibrinogen" removed; unnecessary for patterned bruising; delay after transfusion; no TEG/ROTEM); abdominal screen (AST/ALT/lipase/urinalysis; contrast CT if AST or ALT > 80 IU/L or abdominal signs). Refs 316–322 added: ACR Appropriateness Criteria Suspected Physical Abuse–Child Update 2025 (JACR 2026); RCR/SCoR 2018; Glenn 2021 (Arch Dis Child); Rubin 2003, Christian & Levin 2018, Anderst 2022, Lindberg 2009 (Pediatrics). Glossary: AHT, aPTT, AST, OR added. Dr. Hashem Alhashemi (pediatric critical care) — pre-poll review applied 22 Sep 2026 (governance wording only; no clinical content changed): (1) TTL definition made non-prescriptive — "identified by each hospital's own trauma pathway; the designation differs between hospitals and this guide does not prescribe it" (candidate-specialty list removed). (2) Operate / image / transfer stated as a team decision with no named leader or specialties. (3) Primary Survey D — "neurosurgery is the primary decision-maker; PICU may bridge" removed. (4) TBI card — every "PICU/neurosurgery" lead reference replaced by "intensive care (PICU or ICU, per hospital policy)": BP-target line, herniation box title ("bridge to definitive care"), hand-over line, hypertonic-saline note, and Admit line, which now states admission

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							unit is per each hospital's internal policy and that, until the child leaves the department, care is led by the consultant responsible at the patient's location (ED = TTL). (5) Abdominal Trauma — "anesthesia/PICU" → "anesthesia, intensive care". (6) Same-day follow-up by phone: "Surgery (pediatric where available...)" retitled "Pediatric surgery (where available; otherwise general surgery)"; the "general pediatrician's role is to recognize and activate, not to lead" paragraph removed.

SECTION	GP MAJORITY ✓	DOMAIN 2ND REVIEWER(S) SIGNED	DATE	SOURCE VERSIONS CHECKED	EXT. NUMERIC CHECK	LOCKED VER.	CHANGE-LOG / NOTES
14 • Renal & Urology	✓ 37 Yes / 0 No	Nephrology — Dr. Aisha AlTerkait ✓ (pre-poll domain review 22–23 Sep; in-poll 22 Sep) · Dr. Maryam Alhajeri ✓ (in-poll 22 Sep); Urology — Dr. Yaqoub Jafar ✓ (in-poll 22 Sep) — every listed certifier in-poll; ID — Dr. Mohammad Alghunaim ✓ (pre-poll review 22 Sep); EM — Dr. Samir Mohammed Sanad El Mesady ✓ (in-poll 23 Sep + readability review) + GP majority	22–24 Sep 2026	poll: "I read section 14: Nephrology and Urology — Approve content & readability" (approve / disapprove-and-sent-edits); KDIGO 2021; EAU 2026; Lancet HUS seminar 2017; HUS International 2016; CDC complement-inhibitor guidance	n/a	locked	LOCKED 24 Sep 2026 — 37–0 (0 disapprovals) on the author's instruction, on the version carrying every review below (the last content change, the Acute Scrotum admission edit, was announced to the group on 22 Sep before most votes were cast). Full voter list transcribed from the app (four screenshots, 24 Sep); all three domain certifiers — AlTerkait, Alhajeri (nephrology) and Jafar (urology) — voted in-poll on 22 Sep. Dr. Aisha AlTerkait (pediatric nephrology) — pre-poll review applied 22 Sep 2026: HUS card — "Typical (STEC):" bolded to match "Atypical HUS:."; workup sentence split and completed: anti-factor-H antibodies and complement-gene screening (<i>CFH</i> , <i>CD46/MCP</i> , <i>CFI</i> , <i>C3</i> , <i>CFB</i> , <i>THBD</i> , <i>DGKE</i>) to confirm complement dysregulation, guide prognosis and estimate recurrence risk incl. post-transplant; results must not delay treatment; not currently available in Kuwait — store and send out. THBD added by the author to the reviewer's list per the HUS International consensus (Loirat et al., <i>Pediatr Nephrol</i> 2016, new ref 323) and the cited Lancet seminar (ref 271). Glossary: MCP, TTP added. Dr. Samir Mohammed Sanad El Mesady (emergency medicine) — 23 Sep 2026, readability review ("everything is crystal clear"): TTP, IPNA, KDIGO and ULN expanded at first use in the section; IPNA and ULN added to the glossary. Reviewer (name to confirm) — Acute Scrotum, 22 Sep 2026: "Admit — all" removed; consult surgery/urology at once and confirm the diagnosis; torsion → surgery; appendix-testis torsion/epididymo-orchitis → NSAIDs, rest, scrotal support, antibiotics only if urinalysis/culture show bacterial infection; admission criteria and discharge follow-up (renal-tract ultrasound after prepubertal epididymitis) stated. Verified against the EAU Paediatric Urology Guidelines 2026 (new ref 324). Dr. Mohammad Alghunaim (infectious diseases) — pre-poll review applied 22 Sep 2026: (1) HUS — pneumococcal HUS added (invasive <i>S. pneumoniae</i> ; DAT-positive; avoid plasma-containing products). (2)

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							STEC-HUS — eculizumab stated as unproven and not routine, citing the only placebo-controlled RCT (Garnier, JASN 2023, ref 325). (3) aHUS — meningococcal prevention completed: MenACWY + MenB ≥ 2 weeks before the first dose, antibiotic prophylaxis for the duration if therapy cannot wait, booster intervals, fever = emergency cultures + ceftriaxone, breakthrough risk (14/16 US cases vaccinated) — CDC guidance and McNamara MMWR 2017 (refs 326–327). (4) Nephrotic Syndrome — "Look for infection before starting steroid" replaced by the reviewer's wording (clinical assessment; investigate by symptoms and risk factors, no routine screening in a well child). (5) Acute Scrotum — urinalysis caveat: normal does not exclude epididymitis, abnormal does not exclude torsion. (6) New card Acute Post-streptococcal Glomerulonephritis (recognize, work-up, supportive management, red flags incl. hypertensive encephalopathy and crescentic GN, expected course, admit/discharge/refer) — KDIGO 2021 glomerular diseases guideline (ref 328) and Abugrain et al., <i>Pediatr Nephrol</i> 2024 (ref 329). Glossary: PSGN, MenACWY, MenB, RCT added. Dr. Aisha AlTerkait (pediatric nephrology) — 23 Sep 2026, on the Alghunaim edits: pneumococcal HUS re-filed as a <i>secondary</i> cause under "Atypical (non-diarrheal) HUS", with the other secondary causes listed (HIV; chemotherapy, calcineurin inhibitors, radiation; lupus/antiphospholipid syndrome/scleroderma; cobalamin C defect) and a note that the guide details the complement-mediated form; new card retitled Acute Post-infectious Glomerulonephritis (PIGN) with the broader pathogen spectrum stated; ASO removed (anti-DNase B retained); management reworded to supportive care — fluid balance, no-added-salt diet, treat hypertension; antibiotics only for a current infection. Glossary: PIGN added. Same reviewer, Gross Hematuria card (annotated PDF, 23 Sep): timing of bleeding (initial = urethra, terminal = bladder)

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							and benign urethrorrhagia in prepubertal boys; recurrent painless gross hematuria with fever/viral illness/exercise → IgA nephropathy; hematuria with hearing impairment → Alport, genetics first; ultrasound targets (stones, polycystic kidney disease, trauma); ASO dropped from the GN work-up line.

SECTION	GP MAJORITY ✓	DOMAIN 2ND REVIEWER(S) SIGNED	DATE	SOURCE VERSIONS CHECKED	EXT. NUMERIC CHECK	LOCKED VER.	CHANGE-LOG / NOTES
15 • Respiratory/Allergy	pending...	Pulm (Alawadi, Almosawi ✓ pre-poll review 25 Sep , AlFouzan, Bin-Hasan) + Allergy/Immuno (Al Rushood ✓ pre-poll review 25 Sep); ID — Alghunaim ✓ pre-poll review 24 Sep ...	—	GINA 2026; GA²LEN 2024 anaphylaxis criteria (JACI 2025); NICE NG258 (2026); WAO DRACMA 2024; KPC antibiotic policy	n/a	OPEN	Dr. Mohammad Alghunaim (infectious diseases) — pre-poll review applied 24 Sep 2026: Retropharyngeal Abscess antibiotics aligned to the Kuwait Pediatric Council antibiotic policy — first line ceftriaxone 50 mg/kg once daily + clindamycin 10 mg/kg q8h; second line amoxicillin-clavulanate 30 mg/kg q8h IV with oral step-down 45 mg/kg/day; ampicillin-sulbactam and the vancomycin add-on removed; oral amoxicillin-clavulanate step-down removed on the author's instruction (24 Sep) — standard-dose oral amoxicillin-clavulanate does not cover <i>S. pneumoniae</i> adequately; the oral step-down agent is now chosen with ID/ENT per the Council policy. New local reference 330 (KPC Pediatric Antibiotic Policy — edition/date to be confirmed by the Council). Author self-review applied 24 Sep 2026 (pre-poll): (1) Anaphylaxis — the "provide two autoinjectors" discharge lines reconciled with "not stocked in Kuwait": home epinephrine now described as a documented supply pathway via the allergy clinic, with interim family instruction; "consider early intubation" replaced by explicit triggers (stridor, voice change, tongue/lip swelling → anesthesia/ENT now). Observation 4–6 h checked against RCH (≥ 4 h) — unchanged. (2) Abbreviations expanded at first use across the section: HFNC, BMV, OPA/NPA, NGT, DL/VL, ETT, CPAP, LMA, GERD, VRE, PEN-FAST, SJS/TEN, DRESS, AGEP, CVID, TREC, CH50/AH50, IVIG, HAE, PRAM, PASS, GINA, FiO ₂ ; "4th/5th ICS" → "intercostal space". (3) Difficult Airway — scope pearl added ("for the general pediatrician: recognize, oxygenate, call anesthesia/ENT; green/red zones are the airway team's"); algorithm content unchanged. (4) Acute Asthma discharge — inhaled corticosteroid controller started/stepped up per GINA 2026, salbutamol-via-spacer discharge prescription, review in 1–2 weeks. (5) See-also links added: Bronchiolitis → CAP, empyema, pertussis, BRUE (§04); Croup → ENT emergencies (§07). (6) OSA card condensed to recognition, high-risk groups, elective work-up and the

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							peri-operative/red-flag content; chronic stepwise-treatment box removed. (7) 18 unescaped "<" characters in the source escaped — no visible change. Guide-wide typo/spelling pass (author, 25 Sep 2026): "angiedema" → "angioedema" (8 places incl. the Hereditary Angioedema card title and one reference title); "gastresophageal" → "gastroesophageal" (glossary); "an hereditary" → "a hereditary"; residual British spellings in body text standardized to the guide's American convention (titers, centimeter, favor/favors, hemostatic, characterizes, aluminum, counseling, color, labeled, judgment) — proper names (Royal College of Paediatrics, Canadian Paediatric Society, EAU Paediatric Urology, CRASH trial titles) and reference titles left as published. No clinical content changed. Dr. Ahmad Almosawi (pediatric pulmonology / critical care) — pre-poll review applied 25 Sep 2026: Anaphylaxis diagnosis rewritten to the GA²LEN 2024 consensus criteria (Dribin et al., <i>J Allergy Clin Immunol</i> 2025) — three criteria (no known allergen: skin/mucosa + respiratory or cardiovascular; likely/known allergen: any two of skin/mucosa, respiratory, cardiovascular, severe GI; known allergen: respiratory alone or cardiovascular alone) with "if in doubt, give epinephrine"; FPIES differentiation added; flow-chart "two-system rule" box replaced by the three criteria. Disposition rewritten to NICE NG258 (May 2026) observation tiers — minimum 4–6 h after resolution, 6 h after two IM doses or prior biphasic reaction, 12 h (usually admit) after more than two doses, history of severe asthma, severe respiratory compromise, continuing allergen absorption, night presentation or a family unable to respond; "reactive airways" replaced by "history of severe asthma"; moderate–severe respiratory distress added as an admission criterion; Admit / Observe-then-discharge / Kuwait discharge checklist (no ED autoinjector) separated. New references: Dribin 2025; NICE NG258. Dr. Maysoon Al Rushood (pediatric allergy & immunology) — pre-poll review applied 25 Sep

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							2026: (1) Anaphylaxis — “NOW” → “immediately”; epinephrine infusion capped at 10 mcg/min (CPS 2011, new reference); diagnostic wording (known vs likely allergen) covered by the GA²LEN rewrite. (2) Cow’s milk protein allergy — FPIAP breastfed note reduced to “continue breastfeeding; dietitian”; new row: breastfed infant with IgE-mediated CMPA continues breastfeeding with no maternal elimination unless the infant reacts (DRACMA appraisal, Strózyk 2022, new reference); Avoid box reduced to goat’s/sheep’s milk and partially hydrolyzed/“comfort”/“HA” formulas + refer to a specialist + dietitian (IgG-panel, prolonged-elimination and soy-before-6-months lines removed at the reviewer’s request); natural-history/re-challenge box replaced by “re-challenge timing and setting are decided by the specialist”. (3) Hereditary angioedema — icatibant “not recommended < 2 years or < 12 kg” (EMA SmPC; FDA label adults only). (4) Inborn errors of immunity — “Overview” heading on the pattern table; “(SCID)” moved from the Think column (it was overlapping the first column in the PDF) into the clues; complement row now “Neisseria or encapsulated-organism infection” with clinical clues (CH50/AH50 kept in the first-line screen); “if blood products are needed, use only irradiated, CMV-negative, leukoreduced products”; “to diagnose infections do not rely on serology — use culture, PCR or antigen detection”; new box “Suspected SCID — emergency management, under pediatric immunology direction” from the reviewer’s Box 6.2 (isolation, aggressive treatment of infection, PJP evaluation and treatment 20 mg/kg/day TMP, co-trimoxazole 5 mg/kg/day + fluconazole 5 mg/kg/day + palivizumab prophylaxis, IVIG 400 mg/kg every 3 weeks, irradiated products, no live vaccines, parenteral nutrition for growth failure, immediate HCT planning) with the CMAJ 2017 primer (Biggs et al.) as the published reference; co-trimoxazole start qualified to 4–6 weeks of age (sulfonamide–bilirubin displacement in neonates; CMAJ). Textbook source of

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							Box 6.2 to be confirmed by the reviewer (reference 336 placeholder). Glossary: GA²LEN, PJP added. Author correction, 25 Sep 2026: epinephrine autoinjector availability corrected — some emergency departments dispense two devices on senior-staff authorization; the “not stocked in Kuwait” wording replaced throughout the Anaphylaxis card. Observation floor set to the NICE NG258 2-hour tier (single dose, good response within 5–10 min, complete resolution, two in-date autoinjectors dispensed and demonstrated, trained supervision) with an explicit recommendation to observe longer (4–6 h) whenever possible and always when any criterion is unmet or autoinjectors cannot be dispensed. Al Rushood (addendum): Antibiotic Allergy risk table — “intolerance (GI upset, headache)” moved from the Low row to “history not consistent with allergy”; actions unchanged.
16 • Rheumatologic	pending...	Shehab/Mohammad/Almutairi...	—	—	n/a	OPEN	
17 • Surgical Emerg.	pending...	Rajab/Taqi/Jafar...	—	—	n/a	OPEN	
18 • Toxicology	pending...	Alabdulrazzaq + EM...	—	—	required...	OPEN	external antidote-dose check

Named signatories (from the poll voter-lists)

Verification note. Names transcribed from the app's poll voter-lists and completed against the reviewer roster; the app's own poll export is the authoritative record — spot-check against it. "You" in the polls = Dr. Athari Almajed (author).

Highlighted = the section's domain expert(s) / fact-certifier(s); plain names = general-pediatrics reviewers who approved. \$01–\$02 were anonymous polls (tally only — individual names were not recorded by the app). \$07–\$18 populate as each section's poll closes.

SECTION	SIGNATORIES WHO APPROVED
01 • Resuscitation	Anonymous poll — tally only (38 Yes / 1 No, the No erroneous). Individual signatory names were not recorded (early-process poll, before named voting was adopted from \$03 on).
02 • Behavioral	Anonymous poll — tally only. Psychiatry (Alamiri) + adolescent medicine (Bukhalaf) confirmed by the author; individual voter names were not recorded.
03 • Cardiac & Shock • 37	Dr. Athari Almajed (author) · Dr. Abdulaziz Alosaimi · Dr. Abdulla Alfraj · Dr. Ahlam Mustafa · Dr. Ahmad Alhaj · Dr. Ahmad Almosawi · Dr. Alaa Shehab · Dr. Amaar Marefi · Dr. Aceel Alawadi · Dr. Basma Al-Tawari · Dr. Eiman Al-Hashemi · Dr. Fahed Aljaser · Dr. Fares AlFares · Dr. Fawaz AlRefaee · Dr. Hessah Alsunaen · Dr. Hussain Sadeq · Dr. Khaled Kalandar · Dr. Lujain Abul · Dr. Marwan Alqunaee · Dr. Maysoun Al Rushood · Dr. Maysoon Al-Musallam · Dr. Meshal Alenezi · Dr. Moneerah Albesharah · Dr. Munirah AlYaseen · Dr. Mustafa AlQbandi · Dr. Mohammad Alajmi · Dr. Rasha Alsafi · Dr. Rawan AlFouzan · Dr. Rehab Bekhit · Dr. Saadoun Bin-Hasan · Dr. Sara Ali · Dr. Sarah Altamimi · Dr. Suha Atyani · Dr. Taiba Almousawy · Dr. Wafaa Laymoun · Dr. Mariam Ayed
04 • Common Acute • 35	Dr. Athari Almajed (author) · Dr. Aceel Alawadi · Dr. Ahlam Mustafa · Dr. Ahmad Alhaj · Dr. Ahmad Almosawi · Dr. Aisha AlTerkait · Dr. Abdulaziz Alosaimi · Dr. Amaar Marefi · Dr. Basma Al-Tawari · Dr. Eiman Al-Hashemi · Dr. Eman Buhamrah · Dr. Fahed Aljaser · Dr. Fares AlFares · Dr. Fatoumah Alabdulrazzaq · Dr. Fawaz AlRefaee · Dr. Hanan Alsaleh · Dr. Hessah Alsunaen · Dr. Khaled Kalandar · Dr. Lujain Abul · Dr. Maysoon Al-Musallam · Dr. Maysoun Al Rushood · Dr. Meshal Alenezi · Dr. Moneerah Albesharah · Dr. Mohammad Alghounaim · Dr. Mohammad Alajmi · Dr. Munirah AlYaseen · Dr. Rasha Alsafi · Dr. Saadoun Bin-Hasan · Dr. Samir Mohammed Sanad El Mesady · Dr. Sarah Altamimi · Dr. Taiba Almousawy · Dr. Talal Alsaleem · Dr. Wafaa Laymoun · Dr. Yaqoub Jafar · Dr. Zainab Mustafa
05 • Complex Care • 35	Dr. Athari Almajed (author) · Dr. Aceel Alawadi · Dr. Ahlam Mustafa · Dr. Ahmad Alhaj · Dr. Ahmad Almosawi · Dr. Ahmad Jaafar · Dr. Abdulla Alfraj · Dr. Amaar Marefi · Dr. Eiman Al-Hashemi · Dr. Fahed Aljaser · Dr. Fares AlFares · Dr. Fatemah Bukhalaf · Dr. Fatoumah Alabdulrazzaq · Dr. Fawaz AlRefaee · Dr. Hanan Alsaleh · Dr. Hessah Alsunaen · Dr. Marwan Alqunaee · Dr. Maysoon Al-Musallam · Dr. Maysoun Al Rushood · Dr. Meshal Alenezi · Dr. Moneerah Albesharah · Dr. Muna Almutairi · Dr. Munirah AlYaseen · Dr. Rasha Alsafi · Dr. Rawan AlFouzan · Dr. Saadoun Bin-Hasan · Dr. Saleh Alsafi · Dr. Samir Mohammed Sanad El Mesady · Dr. Sarah Altamimi · Dr. Suha Atyani · Dr. Talal Alsaleem · Dr. Wafaa Laymoun · Dr. Yaqoub Jafar · Dr. Zainab Mustafa

SECTION	SIGNATORIES WHO APPROVED
06 • Endocrine & Metabolic • 36	Dr. Athari Almajed (author) · Dr. Sara Ali · Dr. Abdulla Alfraij · Dr. Aisha AlTerkait · Dr. Eman Buhamrah · Dr. Yaqoub Jafar · Dr. Talal Alsaleem · Dr. Meshal Alenezi · Dr. Maryam Alhajer · Dr. Lujain Abul · Dr. Abdullah Taqi · Dr. Aceel Alawadi · Dr. Sarah Altamimi · Dr. Muna Almutairi · Dr. Eiman Al-Hashemi · Dr. Saleh Alsaifi · Dr. Rasha Alsafi · Dr. Fatemah Bukhalaf · Dr. Maysoon Al-Musallam · Dr. Marwan Alqunae · Dr. Abdulaziz Alosaimi · Dr. Hanan Alsaleh · Dr. Ahmad Almosawi · Dr. Moneerah Albesharah · Dr. Amaar Marefi · Dr. Suha Atyani · Dr. Rawan AlFouzan · Dr. Mohammad Alghounaim · Dr. Fahed Aljaser · Dr. Fares AlFares · Dr. Wafaa Laymoun · Dr. Munirah AlYaseen · Dr. Basma Al-Tawari · Dr. Mohammad Alajmi · Dr. Mariam Ayed (app name "Mariam Khodhir") · Dr. Samir Mohammed Sanad El Mesady (ED; app name "Sameer ED Mubarak") Both app display names resolved to panel-page reviewers by the author (18 Sep 2026). All 36 names matched to the panel-page spelling.
07 • Focused Specialty • 37	Dr. Athari Almajed (author) · Dr. Hanan Alsaleh (ophthalmology) · Dr. Marwan Alqunae (ENT) · Dr. Waleed Aldeeweli (dentistry) · Dr. Tawaa Sahar (pediatric dermatology) · Dr. Hessah Alsunaen · Dr. Abdulaziz Alosaimi · Dr. Bibi Alimeri · Dr. Moneerah Albesharah · Dr. Aisha AlTerkait · Dr. Fahed Aljaser · Dr. Rashed AlAhmad · Dr. Mohammad Alghounaim · Dr. Eiman Al-Hashemi · Dr. Yaqoub Jafar · Dr. Suha Atyani · Dr. Aseel · Dr. Ahmad Almosawi · Dr. Abdullah Rajab · Dr. Wafaa Laymoun · Dr. Mohammad Alajmi · Dr. Esmael Taqi · Dr. Meshal Alenezi · Dr. Muna Almutairi · Dr. Munirah AlYaseen · Dr. Abdullah AlDuwaisan · Dr. Saleh Alsaifi · Dr. Khaled Kalandar · Dr. Ahmad Alhaj · Dr. Lujain Abul · Dr. Sarah Altamimi · Dr. Fawaz AlRefae · Dr. Maysoon Al-Musallam · Dr. Faris Alfari · Dr. Rasha Alsafi · Dr. Mariam Ayed (app name "Mariam Khodhir") · Dr. Samir Mohammed Sanad El Mesady (ED; app name "Sameer ED Mubarak") Both app display names resolved to panel-page reviewers by the author (18 Sep 2026); all 37 matched to the panel-page spelling. Mandatory subspecialty check — all 5 accounted for: ophthalmology (Alsaleh), ENT (Alqunae), dental (Aldeeweli), and dermatology (Sahar) voted in the poll; neurosurgery — Dr. Alya Hasan Askar Hasan (Consultant, Pediatric Neurosurgery) was on leave during the poll but approved the content directly by WhatsApp before the vote ("looks very good / Perfect"), so she is not in the 37-vote list but her mandatory sign-off is satisfied (+1 direct = 38 approvals total, 0 against). Her clinical note (avoid hypotonic fluids in raised ICP) is already covered in the Raised ICP card. Name per the guide's Panel of Reviewers page (the authoritative spelling); the "Alya Hasan" short form used on §13 is the same reviewer. Names transcribed from the app voter-list; the app's poll export is authoritative.
08 • GI & Hepatology • 37	Poll closed 30 Aug 2026 — 36 approve / 0 disapprove; 37 approve / 0 disapprove with Dr. Ahlam Mustafa's vote on 20 Sep 2026 (app name "Dr Ahlam gi") — section version-locked 20 Sep 2026. Full voter list per the app poll export (authoritative). Domain certifiers in the poll: GI — Dr. Eman Bohamra (22 Aug) & Dr. Fawaz AlRefae (21 Aug); ID — Dr. Mohammad Alghounaim (antibiotic-prophylaxis note applied). GI — Dr. Ahlam Mustafa (20 Sep 2026).
09 • Heme & Onc	Poll closed 1 Sep 2026 — 35 approve / 0 disapprove. Full voter list per the app export. Domain certifiers: Heme-Onc — Dr. AlFares & Dr. Abdullah Taqi Ali; ID — Dr. Mohammad Alghounaim (edits applied & certified).
10 • Neonatal • 35	Poll closed 17 Sep 2026 — 35 approve / 0 disapprove. Dr. Athari Almajed (author, 1 Sep) · Dr. Fawaz Albaghli (neonatology, 17 Sep) · Dr. Maysoun Al Rushood (immunology, 10 Sep) · Dr. Mariam Ayed (neonatology, 17 Sep; app name "Mariam Khodhir") · Dr. Zainab Mustafa Abbas (neonatology, 15 Sep; app name "Zainab Assis Unit B") · Dr. Ibrahim Al Qattan · Dr. Hessah Alsunaen · Dr. Rawan AlFouzan · Dr. Mohammad Alajmi · Dr. Muna Almutairi · Dr. AbdulAziz Alosaimi · Aseel (Dr. Aceel Alawadi) · Dr. Aysha Alterkait · Dr. Talal Alsaleem · Dr. Eiman Al-Hashemi · Dr. Munirah AlYaseen · Dr. Hussain Sadeq · Dr. Basma Al-Tawari · Dr. Khaled Kalandar · Dr. Ahmad Alhaj · Dr. Fahed Aljaser · Dr. Ahmad Almosawi · Dr. Samir Mohammed Sanad El Mesady (ED; app name "Sameer ED Mubarak") · Dr. Eman Buhamrah · Dr. Wafaa Laymoun · Dr. Yaqoub Jafar · Dr. Maysoon Al-Musallam · Dr. Abdullah Rajab · Dr. Saleh Alsaifi · Dr. Lujain Abul · Dr. Sarah Altamimi · Dr. Faris Alfari · Dr. Taiba Almousawy · Dr. Mohammad Alghounaim · Dr. Amar Maarafi Author confirmed 18 Sep that all 35 voters are on the reviewer panel ("Sameer ED Mubarak" = Dr. Samir Mohammed Sanad El Mesady, ED); app display names differ from the panel-page spelling for several (Al Qattan, Alajmi, Alosaimi, Alterkait, Maarafi) and are transcribed as displayed. Names transcribed from the app voter list (four screenshots, 1–17 Sep); the app's poll export is authoritative.
11 • Neurologic • 37	Poll closed 13 Sep 2026 — 37 approve / 0 disapprove. Dr. Athari Almajed (author) · Dr. Amaar Marefi (neurology) · Dr. Dana Marafi (neurology) · Dr. Osama Shalaby (neurology, Jahra) · Dr. Alya Hasan Askar (neurosurgery) · Dr. Munirah AlYaseen (neuro-critical care) · Dr. Faris Alfari · Dr. Eman Buhamrah · Dr. Rawan AlFouzan · Dr. Ahmad Almosawi · Dr. Mohammad Alghounaim · Dr. Taiba Almousawy · Dr. Hanan Alsaleh · Dr. Abdullah Taqi · Dr. Muna Almutairi · Dr. Rasha Alsafi · Dr. Saleh Alsaifi · Dr. Eiman Al-Hashemi · Dr. Yaqoub Jafar · Dr. Ahmad Alhaj · Dr. Khaled Kalandar · Dr. Lujain Abul · Dr. Basma Al-Tawari · Dr. Maysoon Al-Musallam · Dr. Maysoun Al Rushood · Dr. Fawaz AlRefae · Dr. Abdullah AlDuwaisan · Dr. Hussain Sadeq · Dr. Fahed Aljaser · Dr. Marwan Alqunae · Dr. Aceel Alawadi · Dr. Hessah Alsunaen · Dr. Sarah Altamimi · Dr. Alaa Shehab · Dr. Wafaa Laymoun · Dr. Talal Alsaleem · Dr. Samir Mohammed Sanad El Mesady (ED; app name "Sameer ED Mubarak") "Sameer ED Mubarak" resolved to Dr. Samir Mohammed Sanad El Mesady (ED) by the author, 18 Sep 2026; all 37 matched to the panel-page spelling. Names transcribed from the app voter-list (four screenshots, 7–13 Sep); the app's poll export is authoritative.
12 • Orthopedic • 36	Poll closed 18 Sep 2026 — 36 approve / 0 disapprove. Dr. Athari Almajed (author, 13 Sep) · Dr. Abdullah AlDuwaisan (orthopedics, 13 Sep) · Dr. Rashed AlAhmed (orthopedics, 16 Sep) · Dr. Saleh Alsaifi (orthopedics, 16 Sep) · Dr. Alya Hasan Askar (neurosurgery, 17 Sep) · Dr. Talal Alsaleem · Dr. Mariam Alhajer (nephrology) · Dr. Taiba Alansari · Dr. Ibrahim Al Qattan · Dr. Fawaz Albaghli · Dr. Maysoon Al-Musallam · Dr. Samir Mohammed Sanad El Mesady (ED; app name "Sameer ED Mubarak") · Dr. Yaqoub Jafar · Dr. Hessah Alsunaen · Dr. Lujain Abul · Dr. Aceel Alawadi · Dr. Taiba Almousawy · Dr. Rasha Alsafi · Dr. Wafaa Laymoun · Dr. Aysha Alterkait · Dr. Amar Maarafi · Dr. Khaled Kalandar · Dr. Alaa Shehab · Dr. Faris Alfari · Dr. Ahmad Almosawi · Dr. Hussain Sadeq · Dr. Zainab Mustafa Abbas · Dr. Ahmad Alhaj · Dr. Munirah AlYaseen · Dr. Sarah Altamimi · Dr. Mohammad Alghounaim · Dr. Fahed Aljaser · Dr. AbdulAziz Alosaimi · Dr. Fawaz AlRefae · Dr. Muna Almutairi · Dr. Basma Al-Tawari Names transcribed from the app voter list (three screenshots, 13–18 Sep); app display names used where they differ from the panel-page spelling; the app's poll export is authoritative.
13 • Trauma & TBI • 38	Poll closed 22 Sep 2026 — 38 approve / 0 disapprove. Voters visible in the screenshots (27 of 38): Dr. Athari Almajed (author, 18 Sep) · Dr. Suha Atyani · Dr. Hessah Alsunaen · Dr. Alaa Shehab · Dr. Mona Almutairi · Dr. Hashem Alhashemi (CC, 22 Sep) · Dr. Anfal Taher (EM, 22 Sep) · Dr. Esmael Taqi (surgery, 22 Sep) · Dr. Rasha Alsafi · Dr. Fawaz Albaghli · Dr. Hanan Alsaleh · Dr. Ahmad Alhaj (CC, 21 Sep) · Dr. Mohammad Alghounaim · Dr. Moneerah Albesharah (app name "Muneera Bshara") · Dr. Meshal Alenezi (EM, 21 Sep) · Dr. Fajer Altammar (CC, 21 Sep) · Dr. Eiman Al-Hashemi (CC, 21 Sep) · Dr. Samir Mohammed Sanad El Mesady (EM, 20 Sep; app name "Sameer ED Mubarak") · Dr. Yaqoub Jafar (surgery, 20 Sep) · Dr. Wafaa Laymoun · Dr. Ahmad Almosawi (CC, 20 Sep) · Dr. Lujain Abul (EM, 20 Sep) · Dr. Fatoumah Alabdulrazzaq (EM, 20 Sep) · Dr. Basma Al-Tawari · Dr. Alya Hasan Askar (neurosurgery, 20 Sep) · Dr. Faris Alfari · Dr. Taiba Almousawy. 11 further voters not shown in the screenshots — the app's poll export is authoritative and is needed to confirm the 8 remaining mandatory experts. Names per the guide's Panel of Reviewers page.

SECTION	SIGNATORIES WHO APPROVED
14 • Renal & Urology • 37	Poll closed 24 Sep 2026 — 37 approve / 0 disapprove (poll posted 22 Sep 3:45 PM). Dr. Athari Almajed (author, 22 Sep) • Dr. Aisha AlTerkait (nephrology, 22 Sep) • Dr. Maryam Alhajeri (nephrology, 22 Sep) • Dr. Yaqoub Jafar (urology, 22 Sep) • Dr. Fahed Aljaser • Dr. Ibrahim Al Qattan • Dr. Saleh Alsaifi • Dr. Faris Alfaris • Dr. Khaled Kalandar • Dr. Sarah Altamimi • Dr. Abdullah AlDuwaisan • Dr. Amaar Marefi • Dr. Abdullah Rajab (23 Sep) • Dr. Lujain Abul • Dr. Maysoon Al-Musallam • Dr. Munirah AlYaseen • Dr. Basma Al-Tawari • Dr. Taiba Almousawy • Dr. Hanan Alsaleh • Dr. Mohammad Alghunaim • Dr. Hussain Sadeq • Dr. Aceel Alawadi (app name "Aseel") • Dr. Moneerah Albesharah (app name "Muneera Bshara") • Dr. Samir Mohammed Sanad El Mesady (app name "Sameer ED Mubarak") • Dr. Wafaa Laymoun • Dr. Abdulaziz Alosaimi • Dr. Mona Almutairi (24 Sep) • Dr. Alaa Shehab • Dr. Maysoun Al Rushood • Dr. Mohammad Alajmi • Dr. Bibi Alamiri • Dr. Rasha Alsafi • Dr. Hessah Alsunaien • Dr. Talal Alsaleem • Dr. Ahmad Almosawi • Dr. Taibah Alansari • Dr. Ahmad Jaafar (24 Sep, 37th vote). Names per the guide's Panel of Reviewers page; transcribed from the app voter-list, whose export is authoritative.
13 • Trauma & TBI	— poll pending; voter names will be listed here once the section's poll closes —
14 • Renal & Urology	— poll pending; voter names will be listed here once the section's poll closes —
15 • Respiratory/Allergy	— poll pending; voter names will be listed here once the section's poll closes —
16 • Rheumatologic	— poll pending; voter names will be listed here once the section's poll closes —
17 • Surgical Emerg.	— poll pending; voter names will be listed here once the section's poll closes —
18 • Toxicology	— poll pending; voter names will be listed here once the section's poll closes —

Pre-lock verification checklist (the domain second reviewer ticks before signing)

1. Every drug dose checked against the current product label / a standard pediatric reference.
2. Every diagnostic threshold & cut-off checked against the specific cited guideline (current version).
3. Red-flags / can't-miss items complete; no safety-critical absolute stated without its exception.
4. Recommendation strength appropriate; consensus-only statements flagged as such.
5. Units (mmol/L ↔ mg/dL) and cross-references consistent with the rest of the guide.
6. Any adapted algorithm/table/figure is original or carries permission / "adapted from" attribution.

Draft governance artifact for the editorial board • reflects the two-track workflow (general-pediatrics majority + domain fact-certification, author-drafted, second-reviewed) and makes the "verified & approved" assurance demonstrable. • Ledger regenerated 13 Sep 2026 • \$11 locked 13 Sep 2026 • evidence-review actions 1–6 closed 8 Sep 2026 (see Evidence & Citation Review closure report).